

Referral Management Policy

Meridian Health Partners: Referral Management Policy

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1. Introduction

1.1. This policy establishes a standardized framework for the processing and management of all patient referrals within Meridian Health Partners (MHP) across its 14 outpatient clinic sites.

1.2. The primary objective is to optimize patient access to specialty care, ensure timely and appropriate care delivery, maintain efficient resource utilization, and enhance communication between referring and consulting providers.

1.3. Adherence to this policy is mandatory for all MHP clinical and administrative staff involved in the referral process.

2. Scope

2.1. This policy applies to all inter-clinic and intra-clinic referrals for MHP patients, whether initiated by MHP Primary Care Providers (PCPs), MHP specialists, or approved external providers.

2.2. It covers the entire referral lifecycle, from initiation and submission to processing, scheduling, and patient notification.

3. Definitions

3.1. Routine Referral: A referral for a non-urgent condition where the patient's health is not immediately at risk and where a delay of several business days will not negatively impact the patient's clinical outcome.

3.2. Urgent Referral: A referral for a condition requiring prompt attention to prevent significant morbidity or progression of disease, but not immediately life-threatening or limb-threatening.

3.3. Emergent Referral: A referral for a life-threatening or limb-threatening condition requiring immediate intervention or evaluation within hours.

3.4. Referring Provider (RP): The MHP clinician initiating the referral (e.g., PCP, specialist).

3.5. Consulting Provider (CP): The MHP specialist receiving the referral for evaluation and treatment.

3.6. Primary Care Provider (PCP): An MHP physician, nurse practitioner, or physician assistant responsible for the comprehensive and continuing care of a patient.

3.7. Referral Coordinator (RC): Designated administrative staff responsible for processing and tracking referrals.

4. Referral Pathways and Requirements

4.1. All referrals, regardless of urgency or specialty, must be submitted via the designated MHP electronic referral management system, unless explicitly stated otherwise in Section 9.0.

4.2. Each referral submission must include: patient demographics, referring provider details, reason for referral, specific clinical question, relevant medical history, current medications, allergies, recent laboratory results, imaging reports, and requested urgency level.

4.3. Direct Referrals (No PCP Sign-off Required):

4.3.1. The following specialties accept direct referrals without requiring a PCP sign-off, provided the referring provider is an MHP clinician:

- * Dermatology (for routine skin checks, suspicious lesions, or chronic skin conditions previously diagnosed by an MHP PCP).
- * Optometry (for routine eye exams, vision correction, or non-emergent eye conditions).
- * Podiatry (for routine foot care, ingrown toenails, or uncomplicated foot pain).
- * Physical Therapy (for initial assessment of musculoskeletal pain lasting less than 6 weeks, or post-operative rehabilitation following MHP surgery).
- * Audiology (for routine hearing assessments or hearing aid evaluations).
- * Behavioral Health (for established MHP patients with a diagnosed mental health condition requiring ongoing therapy or medication management, referred by any MHP clinician).

4.4. PCP-Required Referrals:

4.4.1. All other specialties require an MHP PCP sign-off, indicating that the PCP has performed an initial assessment, documented the clinical necessity, and completed any relevant initial workup (e.g., specific lab tests, imaging, medication trials) before the referral can be processed.

4.4.2. Specialties requiring PCP sign-off include, but are not limited to:

- * Cardiology
- * Gastroenterology
- * Neurology
- * Orthopedics (for initial assessment of complex or chronic musculoskeletal conditions)
- * Endocrinology
- * Pulmonology
- * Nephrology
- * Rheumatology
- * Oncology
- * General Surgery (all types)
- * Urology
- * Ear, Nose, and Throat (ENT)

5. Service Level Agreements (SLAs) for Referral Processing

5.1. All SLAs commence upon the successful receipt of a complete referral by the designated MHP Referral Coordinator.

5.2. An acknowledgment of receipt must be sent to the Referring Provider within 4 business hours of a complete referral being received.

5.3. Routine Referrals:

5.3.1. Definition: Conditions as described in Clause 3.1.

5.3.2. SLA: Within 5 business days, the Referral Coordinator must:

- * Confirm receipt with the referring clinic.
- * Initiate contact with the patient to schedule an appointment.
- * Book the appointment in the MHP scheduling system.

- * Send appointment confirmation to the patient and the Referring Provider.

5.4. Urgent Referrals:

5.4.1. Definition: Conditions as described in Clause 3.2.

5.4.2. SLA: Within 24 hours, the Referral Coordinator must:

- * Contact the Consulting Clinic Lead (e.g., Ms. Brenda Lee, Orthopedics Clinic Coordinator) to flag the urgency.
- * Facilitate direct communication between the Referring Provider and a member of the Consulting Provider's team (e.g., Physician Assistant, RN).
- * Schedule an appointment or initial contact with the patient.
- * Notify the patient and Referring Provider of the confirmed plan.

5.5. Emergent Referrals:

5.5.1. Definition: Conditions as described in Clause 3.3.

5.5.2. SLA: Immediate action, within the same business hour of receipt by the Referral Coordinator:

- * The Referral Coordinator must immediately facilitate direct phone communication between the Referring Provider and the on-call Consulting Provider.
- * The Consulting Provider will determine the immediate course of action (e.g., direct to MHP Emergency Department, immediate clinic slot, direct admission).
- * Patient notification of the confirmed plan must occur within 2 hours of the Consulting Provider's decision.
- * Standard scheduling processes are bypassed to ensure rapid access.

6. SLA Override Procedure

6.1. An SLA may only be overridden in exceptional circumstances, such as critical clinical necessity, unforeseen resource constraints (e.g., unexpected provider leave), or significant system malfunction.

6.2. Authority to override an SLA rests solely with the following individuals, in order of precedence:

- * The Supervising Physician of the Consulting Provider's specialty (e.g., Dr. Aris Thorne, Head of Cardiology).
- * The Medical Director of Meridian Health Partners (Dr. Lena Khan).
- * The Chief Operating Officer of Meridian Health Partners (Mr. David Kim).

6.3. All SLA overrides must be thoroughly documented in the patient's electronic health record (EHR) and the referral management system. This documentation must include:

- * The name and title of the authorizing individual.
- * The specific reason for the override.
- * The original SLA and the revised target timeframe.
- * Any alternative actions taken to mitigate the delay.

7. Escalation Ladder for Unmet SLAs

7.1. Level 1: Initial Delay Identification

- * Role: Referral Coordinator
- * Action: Upon identifying an impending or actual SLA breach (e.g., 24 hours before a Routine SLA expires, or 2 hours before an Urgent SLA expires), the RC must immediately notify the Referring Provider and the Consulting Clinic Lead (e.g., Ms. Brenda Lee, Orthopedics Clinic Coordinator).
- * Timeline: Within 2 hours of identification of the potential breach.

7.2. Level 2: Clinic-Level Intervention

* Role: Consulting Clinic Lead (e.g., Ms. Brenda Lee) and the Referring Provider's Supervising Physician (e.g., Dr. Marcus Chen, Head of Primary Care).

* Action: The Clinic Lead and Supervising Physician will collaboratively assess the bottleneck, explore immediate solutions such as re-prioritizing schedules, contacting other providers within the same specialty, or offering alternative MHP sites.

* Timeline: Within 4 hours of Level 1 escalation.

7.3. Level 3: Departmental Intervention

* Role: Department Head of the Consulting Specialty (e.g., Dr. Aris Thorne, Head of Cardiology) and the Department Head of the Referring Specialty.

* Action: The Department Heads will review the case, allocate high-level resources, consider adjustments to departmental capacity, or approve referral to an external provider if internal capacity cannot meet the demand within the revised timeframe.

* Timeline: Within 8 hours of Level 2 escalation.

7.4. Level 4: Executive Intervention

* Role: Medical Director of Meridian Health Partners (Dr. Lena Khan) or Chief Operating Officer (Mr. David Kim).

* Action: Executive leadership will review systemic issues, make enterprise-level decisions regarding resource allocation or policy adjustments, and ensure appropriate communication with the patient and their family, if necessary.

* Timeline: Within 12 hours of Level 3 escalation.

7.5. Documentation: All steps of the escalation process, including names of individuals involved, dates, times, actions taken, and resolutions, must be meticulously logged in the referral management system.

8. Documented Exceptions to the Standard Pathway

8.1. Exception 1: Established Patient Follow-up (Direct to Specialty)

* Description: A patient who has been seen by a Meridian Health Partners specialist (e.g., Dr. Elena Rossi, Cardiologist) within the last 12 months for the *same specific condition*, and requires a scheduled follow-up.

* Pathway Adjustment: This scenario does not require a new PCP sign-off. The Referring Provider (PCP or another MHP specialist) may directly refer the patient to the previously seen specialist for follow-up. The referral submission must clearly indicate "Established Patient Follow-up" and reference the prior encounter date (e.g., "Follow-up for CHF with Dr. Rossi, last seen 2024-03-15"). This direct referral will follow the "Routine" SLA (5 business days).

8.2. Exception 2: Inter-Specialty Consultation within MHP (Expedited Chart Review)

* Description: An MHP specialist (e.g., Dr. Sarah Bell, Neurologist) requires a formal expert opinion from another MHP specialist (e.g., Dr. Thomas Green, Neurosurgeon) for a complex patient case, based on chart review, without requiring the patient to have an immediate in-person consultation with the second specialist. This is for provider-to-provider guidance.

* Pathway Adjustment: The referring specialist directly submits an "Inter-Specialty Consultation Request" through the electronic system to the consulting specialist's clinic. This request is distinct from a patient referral for an appointment. The consulting specialist's team must acknowledge receipt or provide an initial response within 48 business hours. A formal written opinion or recommendation, based on the provided clinical documentation, must be provided back to the referring specialist within 5 business days of acknowledgment. This does not require a PCP sign-off.

8.3. Exception 3: Emergency Department Direct Admission/Consultation from External Facilities

* Description: A patient presents to an external Emergency Department (ED) and requires immediate direct admission or urgent consultation with a Meridian Health Partners specialist, where utilizing the standard MHP referral system would result in unacceptable clinical delay.

* Pathway Adjustment: The external ED physician directly contacts the MHP on-call specialist via the dedicated MHP-ED hotline. The MHP specialist is obligated to respond to this direct contact within 15 minutes. The MHP specialist will then make a direct clinical decision regarding patient admission or immediate consultation. Formal referral documentation, including patient details and clinical justification, must be completed retrospectively by the ED physician and the MHP specialist's team within 24 hours of the patient encounter. The initial Referral Coordinator intake process is bypassed. This pathway is treated as an "Emergent" referral by the consulting MHP specialist team.

9. Policy Review and Updates

9.1. This policy will be reviewed annually by the Operations Policy Lead and the Medical Director, or more frequently if significant operational changes, regulatory updates, or performance issues warrant.

9.2. Any amendments or updates to this policy must be approved by the Medical Director and Chief Operating Officer of Meridian Health Partners.